



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : هند علي سلام محمد
المحافظة : القاهرة
الحى / المدينة : مصر القديمة
العنوان : *
رقم البطاقة : 27409150104642 رقم التليفون : 01100330574
تاريخ تسجيل الإستمارة : 23-09-2026
العمر (تاريخ الميلاد) : 15-09-1974 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هند علي سلام محمد National ID: 27409150104642	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Name : Hend ali sallam
Branch : Portable
Ref.Physician Prof : cap+c

Date : 21-01-2026 11:36
Medical No. : GRI-27409150104646
Ins.Company :

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION: -**

Findings:

- Bilateral breast outer quadrant soft tissue lesions at right measures 2.8x2.4 cm at left 2.7x2.5 cm with thickened skin more at left 7 mm.
- Bilateral axillary subcentimetric lymph nodes measures at right 6.9 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- T12 lytic bony lesion.

OPINION:

Bilateral breast outer quadrant soft tissue lesions.

- T12 lytic bony lesion for bone scan.

Dr. Mai Seddik

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH
ORAL & I.V. CONTRAST ADMINISTRATION: -**

Findings:

Name : Hend ali sallam

Date : 21-01-2026 11:36

Branch : Portable

Medical No. : GRI-27409150104646

Ref.Physician Prof : cap+c

Ins.Company :

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

PATIENT NAME: HEND ALI SALLAM MOHAMED

GENDER: Female

DATE : 03-Dec-2025 02:13 PM

PATIENT ID: 20225026490

AGE: 51 y

ORDER: Z300460,Z300461

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND**INDICATION:**

- Left breast lump with skin changes.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:**LEFT BREAST:**

- Shrunken breast with global asymmetry associated with diffuse skin thickening as well as nipple retraction.

Complementary ultrasound:

- Left breast is a seat of hypoechoic altered parenchyma mounting to spiculated mass formation, associated with diffuse skin thickening reaching up to 4.5mm.
- The left axilla show multiple indeterminate LNs.
- Free supra- & infra-clavicular lymph nodes.

RIGHT BREAST:

- Scattered fibroglandular parenchyma (ACR B).
- OQs show multiple parenchymal distortion.

Complementary ultrasound:

- OQs show multiple irregular spiculated hypoechoic lesions; the largest about 13.5 x10.5mm & the nearest from the nipple is attached to it & away from the skin about 4mm.
- Normal skin thickness and contour.
- The right axilla show multiple indeterminate LNs.
- Free supra- & infra-clavicular lymph nodes.

OPINION:

- Left breast locally advanced malignant process, as described (BIRADS-5).
- Right breast OQs multiple malignant looking lesions as described (BIRADS-4c).
- For histopathological correlation.



د. يوسف
روستاني
9/16/26



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة الوطنية
للصحة

09/16/2026

MDT FORM

Name:	Hend Aly محمد صالح	MRN:	7299
Age:	52 Y	ID:	27409150104642
Menopausal Sta	prem	Phone:*	01100330574
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		postneoadjuvant	
Surgical History:*	none	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX:

ER: PR: HER2: KI67:

History:

prem bilateral breast cancer rt ct2 no mo multicentric left ct4b no mo ldc
2 er8/8 pr8/8 her2-ve ki 3%
pt rec 4 ac 4 taxotere last 19/8/2026 for surgical referral

Sonomammography:

Biomarker/Molecular

Mets Workup:

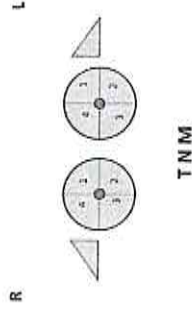
Recommendations:

new sonomammo: 7/9/2026
bilateral nearly stable course

clinically: mild clinical response on lt breast but operable for LT MRM , RT
SM +SLNB

MDT Outcome & Plan

for LT MRM , RT SM +SLNB as per initial decision.



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين	د. لينا عيسى		

16/9/2026
16/9/2026

Patient's Name:

Hind Ali Salam Mohammed

Date:

2026-09-07

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Bilateral pathologically proven breast cancer, right DCIS and left IDC , on NACT assessment study.

Findings:

Right breast.

- Heterogenous dense breast . (ACRc)
- multiple upper outer quadrant irregular shaped dense spiculated mass lesions .
- **Ultrasound** demonstrated nearly stable coarse of of previously seen 11 o'clock irregular anti-parallel heterogenous hypoechoic solid mass lesion with posterior acoustic shadowing measuring about 2.5 cm x 1.9 cm (previously measuring 3.5x 2.5 x2.8) 5 cm from the nipple , multiple innumerable smaller satellite lesions at the UOQ an LQ are still seen at the 9-10 o'clock measuring (9x8mm) previously measuring (12x 10 cm)
- one seen at 12 -1 o'clock zone c deeply seated measuring (1.3 cm x1 cm)
- one seen at 6 o'clock measuring 9x4 mm previously was (11x 8mm)
- Two enlarged level 1 axillary LNs are noted , largest measured 14 x 3mm with 3 mm focal cortical thickness. No pathological infra or supraclavicular axillary lymph nodes.

Left breast.

- Heterogenous dense breast . (ACRc)
- Central breast irregular mass lesion with marked surrounding distortion with .
- **Ultrasound** demonstrated stationary coarse of previously seen mass (previously measuring 7 x3.5 x7.5)altered breast parenchyma involving the whole breast and reaching the nipple mounting to mass formation 6cm x 3cm retroareolar with skin thickening and nipple retraction(locally advanced breast cancer)

- Non-specific axillary LNs. No pathological infra or supraclavicular axillary lymph nodes.

Conclusion:-

- *Right breast (BIRADS 6): naerklly stable course regarding the previously reported malignant process.*
- *Left breast (BIRADS 6): Nearly stable malignant process.*

Best Regards,

DR. Dina SAYED , M.Sc.CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

Suspected Cancer Risk

BIRADS 1: Normal study, routine screening

0 %

BIRADS 2: Benign findings, routine screening

0 %

BIRADS 3: Probably benign, for close follow up.

2 %

BIRADS 4: Probably malignant, for biopsy.

50 %

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

Patient ID: 93339	Patient Name English: Hend Ali Salam Mohamed
المرضى ID: 93339	2026/02/15 11:55:54 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A female patient with pathologically proven invasive lobular carcinoma

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement.

ACR: C (heterogenous dense)

Right breast:

- **Low energy (Non contrast) images:** Right breast shows middle 1/3 UIQ irregular shaped spiculated dense lesion associated with architectural distortion.
- **Recombined (Post contrast) images:**
 - Right breast shows multiple scattered malignant process inform of multiple UIQ and lower quadrants irregular spiculated heterogeneous mild mass enhancement lesions , the largest seen at the UIQ measuring about 3.5x2.5x2.8 cm at its AP, TS and CC diameters and about 8 cm from the nipple and about 2.5 mm from the skin
 - Multiple innumerable small similar satellite are noted at the UIQ and LQs ,
- The most anterior satellite seen parareolar measures about 13x11 mm
- Other lower quadrant multiple enhancing lesion seen, the largest central lower quadrant measuring about 18x14 mm
- **Complementary US:**
 - The right breast shows irregular shaped spiculated hypoechoic lesion located at 11:00 clock position measuring about 2.4x1.7 cm
 - Other small similar satellites are 9:00-10:00 clock position, with retroareolar lesion seen measuring about 12x10 mm
 - One seen at 2 o'clock measuring about 16x9 mm
 - Also noted a 6 o'clock hypochoic irregular mass lesion seen measuring about 11x8 mm
 - A small rather defined 12 oclock lesion seen measuring about 5x3 mm
 - Few enlarged axillary lymph node at level I with focal cortical thickening, measuring about 3 mm

Left breast:

- **Low energy (Non contrast) images:** Left breast central and upper quadrant large density with distortion and nipple retraction And skin traction with thickening
- **Recombined (Post contrast) images:**
 - Large breast ill-defined non mass enhancement measuring about 7x3.5x7.5 cm at its AP, TS and CC diameters from the nipple
- **Complementary US:**
 - The left breast shows large central irregular shaped spiculated hypoechoic lesion located measuring about 7x3 cm
 - Few enlarged axillary lymph node at level I with focal cortical thickening, measuring about 4 mm

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) Right multicentric UOQ, UIQ and LQ enhancing masses as described above.
- **LEFT BREAST:** (BIRADS 6) Large malignant central non mass lesion as described

Best regards,

Dr., Norhan osama, MD